

FAX TRANSMISSION

From: Ridgeview Internal Medicine Associates

To: Johns Hopkins Neurology (general intake?)

Fax: 410-555-8821

Date: 04/18/2026 14:22

Pages: 12 (including cover)

RE: New patient referral – “brain mass???” / headaches (pt denies?)

PLEASE REVIEW AND ADVISE APPROPRIATE CLINIC. PATIENT REQUESTING “2nd opinion neurosurgery vs neurology unsure”

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REFERRAL COVER SHEET

Patient: Kline, Rebecca M

DOB: 09/14/1968

Insurance: CareFirst BCBS (PPO)

Member ID: XJH44290811

Referring Provider:

Daniel P. Hargrove, MD

Ridgeview Internal Medicine

Phone: 703-555-1192

Fax: 703-555-1193

Reason for Referral:

abnormal MRI finding (incidental?) possible tumor vs lesion
evaluate and treat

Notes:

Pt had imaging done for “sinus issues + dizziness” and radiology noted extra-axial lesion. Not clear if symptomatic.

Requested Specialty:

Neurology (or Neurosurgery??)

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PATIENT DEMOGRAPHICS

Name: Rebecca Marie Kline

Address: 14322 Willow Bend Ct

Centreville, VA 20120

Phone: 571-555-6623 (cell)
Alt: 703-555-8822 (home – disconnected per pt)

Sex: Female
Age: 57
Marital Status: Married
Employer: Fairfax County Public Schools (administrative)

Emergency Contact:
Michael Kline (spouse)
Phone: 571-555-8872

Primary Language: English
Race: White
Ethnicity: Non-Hispanic

Allergies:
NKDA (note: earlier chart says “penicillin rash as child” – unclear)

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REFERRING PROVIDER NOTE
Date: 04/15/2026

Chief Complaint:
“follow up MRI results”

HPI:
57 y/o female with history of HTN, hypothyroidism, seasonal allergies presenting after MRI brain obtained by ENT for evaluation of chronic sinus pressure and intermittent dizziness. Imaging reportedly showed “small mass” along left side. Pt denies focal neurologic deficits. No seizures. No vision loss though occasionally “blurry when tired.”

Pt anxious after reading report.

Assessment:
Incidental intracranial lesion (radiology suggests extra-axial).

Plan:
Refer to tertiary center for further evaluation. Consider neurology vs neurosurgical consultation.

Copied forward PMH:
HTN
Hypothyroidism
GERD

Hyperlipidemia
Chronic sinusitis
Vitamin D deficiency

Medications (see separate list – outdated?)

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PRIOR NOTE #1 (Primary Care Visit)
Date: 02/03/2026

CC: sinus pressure, dizziness

HPI:

Pt reports 3–4 months of nasal congestion, worse in mornings. Occasional lightheadedness when standing quickly. No headaches reported today. No nausea/vomiting.

ROS:

Neuro: denies headaches, denies weakness, denies numbness

ENT: + congestion

Cardiac: denies chest pain

Exam:

Neuro: grossly intact

Gait normal

CN II-XII intact

Plan:

Trial fluticasone nasal spray

Refer ENT if no improvement

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PRIOR NOTE #2 (ENT CONSULT)
Date: 03/21/2026

Reason for visit: chronic sinusitis

HPI:

Pt describes “pressure headaches” (contradicts PCP note?) mostly frontal. Also occasional dizziness. No syncope.

Nasal endoscopy: mild mucosal edema

Assessment:
Chronic rhinosinusitis
Headache – likely sinus related

Plan:
MRI brain/sinuses to evaluate further given atypical dizziness

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PRIOR NOTE #3 (Urgent Care Visit)
Date: 01/11/2026

CC: headache

HPI:
Pt presents with dull headache x2 days. Rates 4/10. No photophobia. No neuro deficits. Reports similar headaches “on and off for years.”

Exam:
Neuro nonfocal

Assessment:
Tension headache

Plan:
Ibuprofen PRN
Hydration

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IMAGING REPORT

Facility: Fairfax Imaging Center
Date: 04/02/2026

MRI BRAIN WITH AND WITHOUT CONTRAST

Findings:
There is a well-circumscribed extra-axial lesion along the left parietal convexity measuring approximately 1.8 x 1.5 x 1.3 cm. The lesion demonstrates homogeneous enhancement. There is a suggestion of a dural-based attachment. No significant surrounding edema. No midline shift.

Ventricles normal in size. No acute infarct.

Impression:

Enhancing extra-axial lesion, likely benign in etiology. Differential considerations include meningeal-based lesion. Recommend clinical correlation and follow-up imaging.

Incidental: mild sinus mucosal thickening

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LAB RESULTS (partial, mixed dates)

CMP 03/28/2026

Na 139

K 4.2

Cl 101

CO2 26

BUN 14

Cr 0.8

CBC 03/28/2026

WBC 6.2

Hgb 13.4

Plt 241

TSH 02/03/2026

TSH 3.8

Lipid Panel 01/10/2026

Total Chol 212 (H)

LDL 138 (H)

Vitamin D 01/10/2026

25-OH Vit D 22 (low)

Urinalysis (unrelated?) 12/2025

normal

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MEDICATION LIST (EHR auto-generated, may not be current)

Lisinopril 10 mg daily

Levothyroxine 75 mcg daily

Omeprazole 20 mg daily

Atorvastatin 20 mg nightly

Fluticasone nasal spray PRN
Vitamin D3 2000 IU daily
Ibuprofen 200 mg PRN
Multivitamin daily
Melatonin 5 mg PRN sleep
Cetirizine 10 mg daily

Note: pt states she “sometimes forgets statin”

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DISCHARGE SUMMARY (ENT OUTPATIENT)
Date: 04/02/2026

Procedure: MRI completed

Course:
Pt tolerated imaging without complication.

Instructions:
Follow up with PCP for results
Return if worsening headaches, vision changes, weakness

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PROBLEM LIST (copied forward, not reconciled)

Hypertension
Hypothyroidism
Hyperlipidemia
GERD
Chronic sinusitis
Headache disorder (unclear type)
Dizziness
Vitamin D deficiency
Obesity (BMI 29.8 – borderline?)

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MISC DOCUMENT (partial printout, truncated)

...patient reports no seizures or loss of consciousness. Family history notable for mother with “brain tumor?? benign per pt” unclear details.

Social history:

Never smoker

Alcohol: occasional wine

No illicit drug use

...line cut off...

END OF FAX TRANSMISSION